

China Pharma and Biotech

China Pharma & Biotech: ASCO 2026 Part 3 - Summary (for when fundamentals begin to matter again)



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Post-ASCO, healthcare stocks have taken the worst hit in recent history. Multiples are at or below their Apr 2024 trough. We don't see any fundamental negatives that could explain the capital outflow, and at this point it's certainly beyond fundamentals. Nonetheless, here is a summary of post-ASCO views while waiting for the tide to turn.

Kelun-Biotech (O): sac-TMT emerges as the most credible IO partner in lung cancer (see [Part 1](#) of the series). Kelun's presentation strengthened the story with **higher ORR, deeper tumor shrinkage, and more durable responses** with sac-TMT+pembro than with pembro alone. It's therefore more likely that the strong OS benefits would hold true towards maturation. Beyond 1L, sac-TMT is differentiated in 2L+ EGFR-mutant NSCLC, where it remains the only agent with statistically significant OS HR. And the recent first global p3 win in late-line endometrial cancer, announced by partner Merck (covered by Courtney Breen), adds important validation ex-China. Kelun's pipeline extends beyond sac-TMT: SKB500 (B7H3 ADC) showed a differentiated safety profile in 2L+ SCLC.

Kelun is down 15% since Monday (YTD flat). We see in this name **the biggest imbalance between fundamentals** (strong incremental gain since ASCO, no over-hang) and the top opportunity when momentum reverses.

Innovent (O): IBI363 plays in the **less crowded segment of I/O-treated patients and shows a striking survival tail**: 24-month OS of 48% in squamous and 43% in adenocarcinoma, versus 8% and 21.7% for docetaxel, and 29% for Dato-DXd in adeno. In 1L patients, IBI363 showed the highest ORR 86% across trials. The PD-1 $\times$ IL-2 β bispecific design could ultimately challenge not only current PD-1 monotherapies but also the first wave of PD-1 $\times$ VEGF bispecifics.

Innovent is down 8% since Monday (YTD flat). This is another **imbalance with improving prospect on IBI363 after ASCO and two mega-deals announced this year** (with LLY and PFE, covered by Courtney Breen). Risks on mazdutide sales should also be contained (see our GLP1 sales tracker). **Innovent is due to report updates on its new GLP-1 asset (including oral GLP-1 FIH data) at the upcoming ADA.** We see Innovent as the other idiosyncratic opportunity besides Kelun.

Akeso (MP): Despite the pointed commentary given by ASCO discussant, the strong OS HR is a first-time achievement for AK112, any PD-1 $\times$ VEGF, or any approach involving both I/O and VEGF mechanisms in this setting ever. We believe there is **positive readthrough to the global HARMONI-3-sq** that will release its final PFS and interim OS readout in 3Q26. Meanwhile, results from H-2 (insignificant OS, sq and nsq) and H-6 (significant OS, sq only), when taken together, do not give us more confidence on HARMONI-3-nsq - it would still be an uphill battle to win the larger histology.

We also summarized positive outcomes (albeit early) from Hengrui's Nectin-4 ADC and BeOne's CDK4i (first-in-human 1L) here.

BERNSTEIN TICKER TABLE

Ticker	Rating	2 Jun 2026		Price Target	TTM Rel. Perf.	Reported EPS				Reported P/E (x)		
		Cur	Closing Price			Cur	2025A	2026E	2027E	2025A	2026E	2027E
9926.HK (Akeso)	M	HKD	106.50	130.00	(18.8)%	CNY	(0.60)	(0.52)	0.70	(152.0)	(178.1)	131.5
ONC (BeOne)	O	USD	274.69	412.00	(16.5)%	USD	2.63	5.65	8.93	104.4	48.6	30.8
1093.HK (CSPC)	M	HKD	7.57	10.70	(54.1)%	HKD	0.37	0.52	0.56	17.7	12.7	11.6
3692.HK (Hansoh)	O	HKD	31.02	47.00	(33.7)%	CNY	0.93	1.01	1.10	28.8	26.5	24.3
1801.HK (Innovent)	O	HKD	78.85	120.00	(29.6)%	HKD	0.48	0.71	2.90	162.6	111.1	27.2
600276.CH (Hengrui)	O	CNY	48.40	71.00	(62.6)%	CNY	1.19	1.54	1.92	40.7	31.3	25.2
6990.HK (Kelun-Biotech)	O	HKD	403.60	526.00	(32.2)%	CNY	(1.66)	(2.75)	0.85	33.4	34.0	20.9
1177.HK (Sino BioPh)	M	HKD	4.88	7.90	(41.9)%	CNY	0.19	0.20	0.22	22.0	21.2	19.2
9688.HK (Zai Lab)	M	HKD	13.60	15.00	(94.5)%	USD	(0.16)	(0.15)	(0.09)	0.4	0.4	0.3
ASIA			2,056.69									
SPX			7,609.78									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

6990.HK, 9688.HK valuation is EV/Sales (x); 9926.HK, 1093.HK, 1177.HK base year is 2024;

Source: Bloomberg, Bernstein estimates and analysis.

DETAILS

SECTOR PERFORMANCE

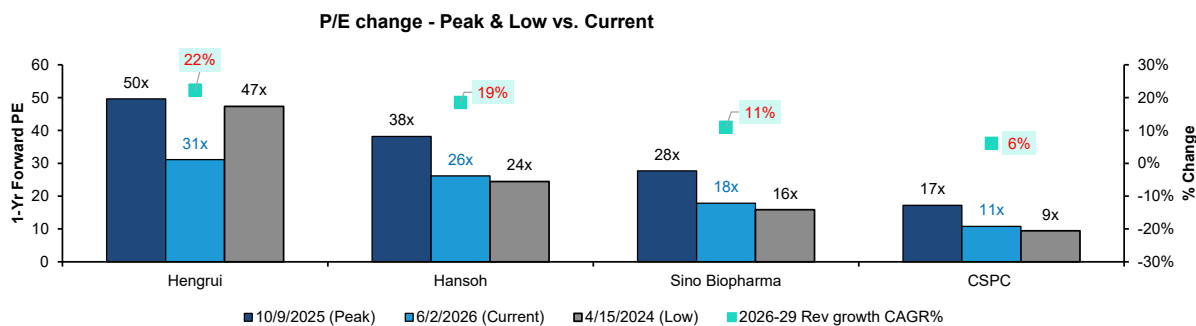
CURRENT ROUT VS. RECENT PEAK (2025) AND TROUGH (2024)

EXHIBIT 1: Sector performance has been fluctuating and declining since 2026



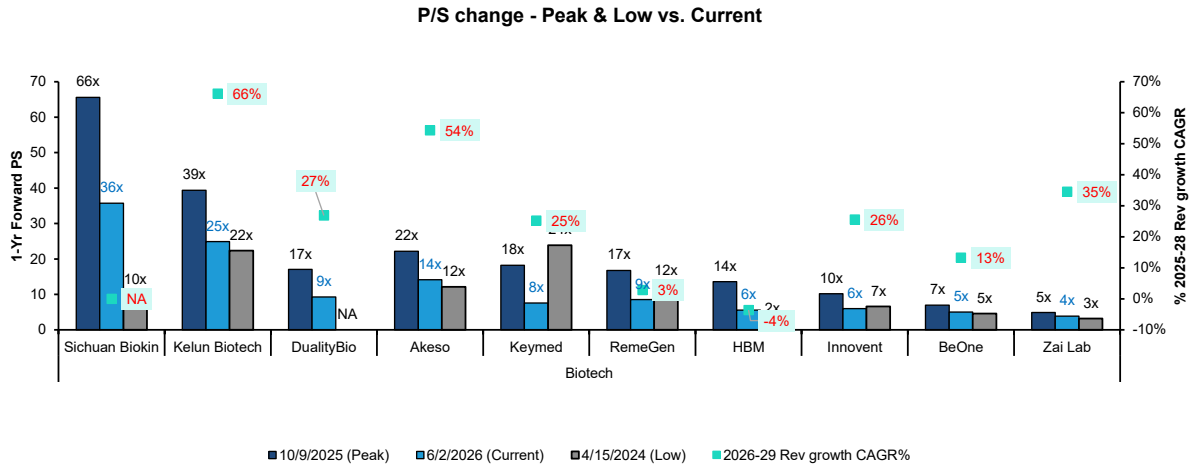
Source: Bloomberg, Bernstein analysis

EXHIBIT 2: In terms of 1-yr forward P/E, pharma valuation has nearly reached all-time low



Source: Bloomberg, Bernstein analysis

EXHIBIT 3: In terms of 1-yr forward P/S, China biotech companies have seen a decline since sector peak in 2025



Note: Duality Bio, Keymed, RemeGen, and HBM are not covered
Source: Bloomberg, Bernstein analysis

ASCO REVIEW

LUNG CANCER: THREE ASSETS PLAY IN A \$30 BN TAM

Akeso's AK112 secures OS win but still fights an uphill battle; Kelun's sac-TMT faces the lowest commercial hurdle; Innovant's IBI363 still early days, but differentiates with long survival in 2L+ and highest response rate in 1L

[Ivonescimab plus chemotherapy versus tislelizumab plus chemotherapy in previously untreated advanced squamous non-small cell lung cancer: Overall survival results of the phase 3 HARMONI-6 trial. - ASCO](#)

[Updated results from a phase 2 trial of SSGJ-707 \(PF-08634404\), a PD-1/VEGF bispecific antibody, as monotherapy in patients with advanced non-small cell lung cancer \(NSCLC\). - ASCO](#)

[Phase 2 data from ROSETTA Lung-02, a global randomized phase 2/3 trial of pumitamid \(PD-L1 x VEGF-A bsAb\) + chemotherapy in 1L NSCLC. - ASCO](#)

[SHR-A2102 in combination with adebrelimab as first-line treatment in patients with locally advanced or metastatic squamous or non-squamous non-small cell lung cancer \(NSCLC\): Results from a phase 1b/2 study. - ASCO](#)

Our note last week highlighted the significant outcome of Kelun's sac-TMT in 1L PD-L1+ NSCLC. In the PD-L1 all-comer population, different strategies are showing meaningful breakthroughs.

PD-1 x VEGF: In the HARMONI-6 trial, AK112 has lifted the mOS to 27.0 months, with an OS HR of 0.66 compared to the control group Tislelizumab + chemo. Across the PD-1 x VEGF therapies ORR has been consistently > 70%. This is in sharp contrast to the earlier HARMONI-2 trial where a strongly positive PFS HR had not translated into statistically significant OS at the interim analysis. There was also a pointed commentary given by the plenary discussant that challenged the trial based on its age and gender mix as well as exclusion based on bleeding risks (see [report](#)). Nonetheless, the strong OS HR is a first-time achievement for AK112, any PD-1xVEGF, or any approach involving both I/O and VEGF mechanisms in this setting ever. We believe there is positive readthrough to the global HARMONI-3-sq that will release its final PFS and interim OS readout in 3Q26. Meanwhile, results from H-2 (insignificant OS, sq and nsq) and H-6 (significant OS, sq only), when taken together, do not give us more confidence on HARMONI-3-nsq - it would still be an uphill battle to win the larger histology.

sac-TMT: We noted in PART1 of the ASCO series that OptiTROP-Lung05 results heralded sac-TMT as the best booster of existing I/O (Keytruda), given its strongly positive PFS HR 0.35 (implying over 16m PFS) and OS HR trending at 0.55. Kelun's presentation further showed that sac-TMT combined with pembrolizumab solicits higher ORR, deep response rate, and duration of

response compared to pembro monotherapy. These data further support the case for a positive OS ultimately.

PD-1 x IL-2: IBI363 achieved highest ORR (85.7%) across the groups, but PFS and OS still immature. As IBI363 already shows favorable survival tail in I/O-treated patients (see below), we see more potential for the bispecific to challenge existing I/O and PD-1XVEGFs.

Nectin-4: Traditionally nectin-4 ADCs have been used more on mUC type of cancer. However, in the data published by Hengrui on NSCLC, an ORR of 80.8% was reported - also at the upper end, but more data on DoR and survival would be necessary before we get too excited for this more unorthodox approach.

EXHIBIT 4: Global NSCLC market opportunity analysis: 1L & 2L+ segments (TAM, incidence, and treatment)

FIRST LINE (1L) NSCLC					SECOND LINE (PLUS) NSCLC						
	% of total 1L NSCLC	Incidence	TAM (\$ Bn)	Current SoC	Emerging therapies		% of total 2L+ NSCLC	Incidence	TAM (\$ Bn)	Current SoC	Emerging therapies
Total 1L	100%	<2 Mn	25-30			Total 2L+	100%	<1 Mn	~15		
AGA ^a	35-45%	550-720k	10-15			AGA ^a	30-40%	250-350k	~6		
EGFR	15-20%	240-320k	7-8	Target therapies	EGFR-TKIs	EGFR	15-20%	120-160k	3-4	Chemo; EGFR×c-Met bispecific	- TROP2 ADC - PD-1×VEGF + chemo
KRAS G12C	7-12%	110-190k	1-2		KRAS	7-12%	60-80k				
ALK	3-7%	100-120k	3-5		ALK	3-7%	70-110k	2-3	Target therapies		
Others (MET, ROS, BRAF, etc.)	3-5%				Others (MET, ROS, BRAF, etc.)	3-5%					
Non-AGA	55-65%	700-900k	15-20			Non-AGA	60-70%	500-600k	~9		
PD-L1 <1%	33% of non-AGA	260k	~8	PD-(L)1 with or without chemo	- PD-1×VEGF (+chemo)	Non-squamous	63% of non-AGA	350k	~6	Chemo (DTX ^b / PTX ^b)	- PD-1×IL-2
PD-L1 1-49%	37% of non-AGA	300k	~6		- PD-1 + TROP2 ADC	Squamous	37% of non-AGA	200k	~3	Chemo (DTX / GC ^b)	
PD-L1 >49%	30% of non-AGA	240k	~6		- PD-1×IL-2 (+chemo)						
1L emerging therapies: - So far AK112 has shown in PFS and OS (sq only) benefits - broader applicability across pt populations to be proven; - sac-TMT shows the best add-on effect to existing Keytruda SoC; - IBI363 has solicited the highest response rates agnostic to PD-L1 or sq/nsq						2L+ emerging therapies: - Sac-TMT shows compelling data in EGFRm pts; AK112 has not achieved OS endpoint - IBI363 differentiates as the leader in non-AGA pts for whom few options beyond chemo available					

Note: a. Almost all AGAs are found in adenocarcinoma (non-squamous); b. DTX = Docetaxel, PTX = Pemetrexed, GC = Gemcitabine

Source: ASCO, ESMO, NIH, PubMed, Nature, Bernstein analysis and estimates

EXHIBIT 5: Comparison of leading domestic assets in NSCLC: Clinical data & sales outlook

Company	Asset	Clinical data overview		Trial development status		Market & sales potential		
		1L key data	2L+ key data	No. global Ph3s completed	No. global Ph3s in progress	TAM in NSCLC (\$ Bn)	Est. peak sales in NSCLC post POS (\$ Bn)	Total peak sales (\$ Bn)
Akso	AK112	HARMONI-2 : interim OS HR = 0.777 vs. Keytruda (not S.S.); interim PFS HR = 0.51. HARMONI-6 : interim OS HR = 0.66 vs. tis + chemo; interim PFS HR = 0.60.	HARMONI (EGFRm) : Final OS HR = 0.78 vs. chemo (LTF); final PFS HR = 0.57 (LTF).	1	4 (3 in lung)	20-25	4.0	6.8
Kelun-Biotech	sac-TMT	OptiTROP-Lung05 : interim OS HR = 0.55 vs. Keytruda; interim PFS HR = 0.35.	OptiTROP-Lung04 (2L EGFRm) : interim OS HR = 0.60 vs. PBC; interim PFS HR = 0.49. OptiTROP-Lung03 (2L+ EGFRm) : interim OS HR = 0.49 vs. PBC; interim PFS HR = 0.30.	1	16 (5 in lung)	20-25	3.0	6.6
Innovent	IBI363	Ph1^b : cORR 81.8% (ORR 85.7% in sq, ORR 87.5% in nsq)	Ph1 (non-AGA)^c : mPFS = 10.1m (sq) & 4.2m (nsq); mOS = 18.2m (sq) & 15.2m (nsq); 24-m OS rate = 47.8% (sq) & 42.7% (nsq).	0	1 in lung	25-30 ^b	1.8	3.1

Note: a. Innovent has higher TAM in lung because of its differentiation in I/O-treated patients; b. Efficacy from 3-1.5mg/kg dose level cohort; c. Efficacy from 3mg/kg dose level cohort

Source: Company disclosures, ASCO, WCLC, ClinicalTrials.gov, Bernstein analysis and estimates

EXHIBIT 6: In 1L PD-L1 all comer sq-NSCLC settings, AK112 + chemo demonstrated a strongly positive and S.S. OS HR of 0.66 vs. tisle + chemo, along with a numerically longer median OS

						ASCO 2026				
	PD-(L)1 mAb				PD-(L)1 BsAb				ADC	
	Pembrolizumab		Tislelizumab		AK112		Pumitamig	SSGJ-707	IBI363	SHR-A2102
Sponsor	Merck		BeOne		Akeso		BioNTech	3S Bio	Innovent	Hengrui
Trial	KEYNOTE-407		RATIONALE 307, NCT03594747		HARMONI-6, NCT05840016		ROSETTA Lung-02, NCT06712316	NCT06361927	NCT06468098	NCT06512051
Target modality	PD-1 mAb		PD-1 mAb		PD-1 × VEGF-A BsAb		PD-L1 × VEGF-A BsAb	PD-1 × VEGF-A BsAb	PD-1 × IL-2 ^{2-bias} BsAb fusion protein	Nectin 4 ADC
LoT	1L		1L		1L		1L	1L	1L	1L
PD-L1 level	PD-L1 all-comer		PD-L1 all-comer		PD-L1 all-comer		PD-L1 all-comer	PD-L1 TPS≥1%	PD-L1 all-comer	PD-L1 all-comer
Intervention (arm)	Pembro + chemo	PBO + chemo	Tislelizumab + PC	PC	AK112 + chemo	Tislelizumab + chemo	pumitamig + chemo	SSGJ-707 10mg/kg	IBI363 3-1.5 mg/kg ^b + PDC	SHR-A2102 + adebrelimab
Efficacy evaluable pts	278	281	120	121	266	266	19	12	14	26
ORR%	62.6%	38.4%	74.2%	47.9%	75.9%	66.5%	73.7%	75.0% ^a	85.7%	80.8%
mPFS (months)	8.0	5.1	7.7	5.5	11.1	6.9		8.9	Immature (events 28.8%)	
PFS HR	0.57		0.45		0.6					
mOS (months)	17.1	9.1 (adj.) 11.6 (unadj.)	22.8	20.2	27.9	23.7		NR (4.9-NR)		
OS HR	0.59		0.68 ([0.46, 1.01])		0.66 ([0.50, 0.87], p = 0.0017)					

Note: a. Confirmed ORR; b. 3-1.5mg/kg represents IBI363 3 mg/kg plus PDC in cycle 1, then 1.5 mg/kg Q3W plus PDC in subsequent cycles; Merck is covered by Courtney Breen, BioNTech is covered by Jeffrey Walch, 3S Bio is not covered

Source: ASCO, NEJM, ClinicalTrials.gov, Bernstein analysis

EXHIBIT 7: In 1L PD-L1 all comer nsq-NSCLC settings, Innovent's IBI363 + chemo combo also demonstrates the highest ORR vs. peers

			ASCO 2026			
	PD-(L)1 mAb		PD-(L)1 BsAb			ADC
	Pembrolizumab		IBI363	Pumitamig	SSGJ-707	SHR-A2102
Sponsor	Merck		Innovent	BioNTech	3S Bio	Hengrui
Trial	KEYNOTE-189, NCT02578680		NCT06468098	ROSETTA Lung-02, NCT06712316	NCT06361927	NCT06512051
Target modality	PD-1 mAb		PD-1 × IL-2 ^a -bias BsAb fusion protein	PD-L1 × VEGF-A BsAb	PD-1 × VEGF-A BsAb	Nectin 4 ADC
LoT	1L		1L	1L	1L	1L
Histology	nsq-NSCLC		advanced NSCLC (nsq subgroup)	advanced NSCLC (nsq subgroup)	advanced NSCLC (nsq subgroup)	advanced NSCLC (nsq subgroup)
Intervention (arm)	Pembro + chemo	PBO + chemo	IBI363 3-1.5 mg/kg ^b + PDC	pumitamig + chemo	SSGJ-707 10mg/kg	SHR-A2102 + adebrelimab
Efficacy evaluable pts	410	206	8	21	22	33
ORR%	48.0% ^a	19.4% ^a	87.5%	66.7%	63.6% ^a	69.7%
mPFS	9.0	4.9	Immature (events 28.8%)		12.4	
PFS HR	0.48				NR (14.8-NR)	
mOS	22.0	10.7				
OS HR	0.56					

Note: a. Confirmed ORR; b. 3-1.5mg/kg represents IBI363 3 mg/kg plus PDC in cycle 1, then 1.5 mg/kg Q3W plus PDC in subsequent cycles

Source: ASCO, NEJM, ClinicalTrials.gov, Bernstein analysis

EXHIBIT 8: ORR, PFS HR, and OS HR of the key trials under watch

Subgroup analysis	PD-L1 expression			Histology	
	TPS < 1%	TPS 1-49%	TPS ≥ 50%	Squamous	Non-squamous
ORR, %					
IBI363: NCT06468098 (IBI363 + PDC) (cORR)	76.9%	87.5%		78.6%	87.5%
HARMONi-6 (ivo + chemo vs tis + chemo) (ORR)	69.5%	80.1%			
PFS HR					
HARMONi-2 (ivonecimab vs pembro)		0.54	0.46	0.48	0.54
OptiTROP-Lung05 (sac-TMT+pembro vs pembro)		0.28	0.47	0.44	0.28
OS HR					
HARMONi-6 (ivo + chemo vs tis + chemo)	0.64 (0.43-0.96)	0.67 (0.42-1.05)	0.64 (0.32-1.31)		

Source: Lancet, WCLC, ASCO, Company disclosure, Bernstein analysis

EXHIBIT 9: At 2026 ASCO, HARMONi-6 published updated safety profile with increased irAE and VEGF-related TRAE, but overall still tolerable

Target & modality	PD-(L)1 mAb		PD-(L)1 BsAb					ADC
Drug	Pembrolizumab ^b		AK112 + chemo	Tis + chemo	IBI363 ^a	Pumitamig	SSGJ-707	SHR-A2102
Trial	KEYNOTE-189/407		HARMONI-6		NCT06468098	ROSETTA-Lung02	NCT06361927	NCT06512051
Histology	sq NSCLC	nsq NSCLC	sq NSCLC		sq & nsq NSCLC	sq & nsq NSCLC	sq & nsq NSCLC	sq & nsq NSCLC
Any grade TRAEs	98.6%	99.8%	99% (-)	99% (↑1%)		93.0%	92.8%	
Immune-related AEs (irAEs)	35.3%	26.4%	27.4% (UD)	25% (UD)		14.0%	37.3%	
VEGF-related AEs			60% (↑14%)	26% (↑3%)			62.7%	
Decreased neutrophil count	37.8%	27.7%	47% (↑2%)	45% (↑2%)	75.0%			
Anemia	54.7%	47.4%	57% (↑4%)	61% (↑3%)	78.8%			
Leukopenia / WBC decreased			38% (↓12%)	37% (↑3%)	63.8%			
Grade ≥3 TRAEs	74.1%	71.9%	69% (↑5%)	59% (↑5%)	81.3%*	44.2%	42.2%	53.7%
Immune-related AEs (irAEs)	13.3%	10.9%	14% (↑5%)	14% (↑4%)		2.3%	8.4%	
VEGF-related AEs			12% (↑5%)	3% (↑1%)			18.1%	
Decreased neutrophil count	23.0%	16.0%	32% (-)	26% (-)	42.5%			32.8%
Anemia	15.8%	18.3%	7% (↓1%)	5% (↑1%)	18.8%			9.0%
Leukopenia / WBC decreased			11% (↓5%)	9% (-)	20.0%			14.9%
TRAEs led to treatment discont. or death	27.3%	40.7%	disc. 5% (↑2%) death 4% (↑1%)	disc. 5% (↑1%) death 4% (-)	7.6%	4.7%		

Note: a. TEAEs; b. All-Cause Adverse Events; * 65.2% for 3-1.5 mg/kg; UD: undisclosed at ASCO 2026 paper

Source: Lancet, ASCO, NEJM, ClinicalTrials.gov, Bernstein analysis

EXHIBIT 10: In 1L PD-L1 positive NSCLC setting, sac-TMT + pembrolizumab delivered 70.2% of ORR, with PFS HR at 0.35 and OS HR at 0.55, competitive across groups

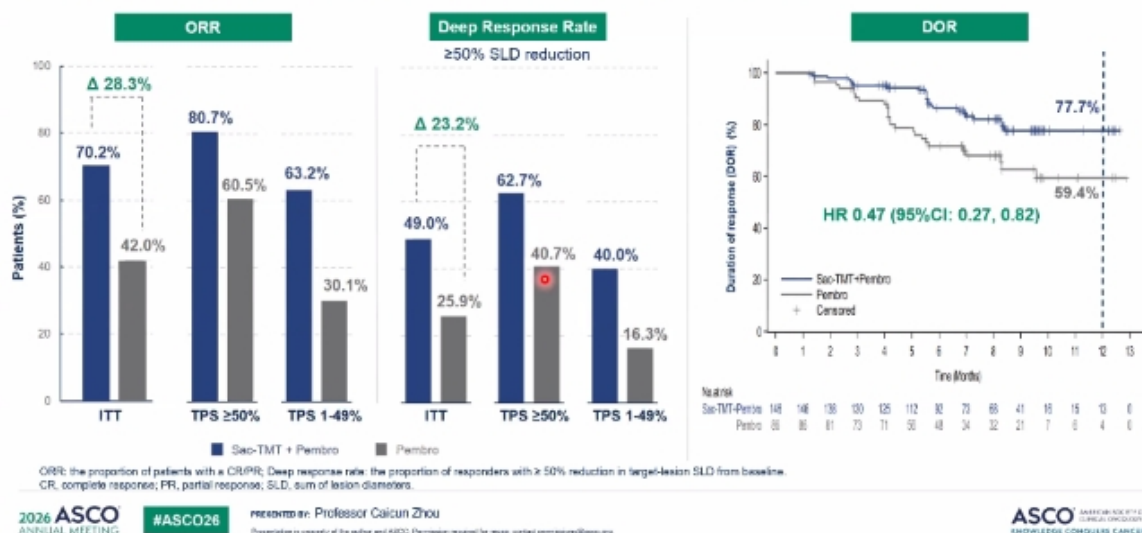
	PD-1/L1 monotherapy		PD-1 + chemo			IO + Anti-angiogenic		TROP2 ADC + PD-1						
	Pembrolizumab		Pembrolizumab + chemo			Ivonescimab		Sac-TMT		Trodelyv		Dato-DXd		
Sponsor	Merck		Merck		Merck		Akeso / Summit		Kelun-Biotech		Gilead		DS / AZ	
Trial	KEYNOTE-042 (NCT02220894)		KEYNOTE-407 (sq-NSCLC)		KEYNOTE-189 (nsq-NSCLC)		HARMONI-2 (NCT05499390)		OptiTROP-Lung05		EVOKE-02 (NCT05186974)		TROPION-Lung02	
Target modality	PD-1		PD-1		PD-1		PD-1/VEGF		PD-1 + TROP2 ADC		PD-1 + TROP2 ADC		PD-1 + TROP2 ADC	
PD-L1 expression	PD-L1 TPS ≥1%		PD-L1 TPS ≥1%		TPS 1-49% TPS ≥ 50%		PD-L1 TPS ≥1%		PD-L1 TPS ≥1%		TPS 1-49% TPS ≥ 50%		TPS < 50% TPS ≥ 50%	
Intervention (arm)	Pembrolizu mab	Chemo	Pembrolizumab + chemo		Pembrolizumab+chemo		Ivonescimab	Pembrolizum ab	Sac-TMT + pembrolizumab	Pembrolizumab	Trodelyv + pembrolizumab		Dato-DXd + pembrolizumab	
n	637	637	176		186 202		198	200	208	205	33	30	30	5
ORR%	27.3%	26.7%	59.1%		49.2% 62.1%		50%	39%	70.2%	42.0%	36.4%	66.7%	50%	100%
mPFS	5.6	6.8	8.0		9.2 11.1		11.1	5.8	NR at mFU 10.5m	5.7	9.1	13.1	11.1	> 8.3, NE
PFS HR	1.05						0.51		0.35 (95% CI 0.26 - 0.47)					
mOS	16.4	12.1	18.9		21.8 > 20.4				Not mature				> 19.2, NE > 12.6, NE	
OS HR	0.79						0.777		0.55 (95% CI 0.36 - 0.85)					
Safety	G≥3 TRAE: 18%	G≥3 TRAE: 41%	G≥3 TRAE: 56.5% (all patients)		G≥3 AE: 71.9% (all patients)		G≥3 TRAE: 29.4%	G≥3 TRAE: 15.6%	G≥3 TEAE: 55.3%	G≥3 TEAE: 31.4%	G≥3 TEAE: 76.1%		G≥3 TRAE: 40.5%	

Source: IASLC, WCLC, J Clin Oncol, ASCO, Bernstein analysis

EXHIBIT 11: In PD-L1+ NSCLC (1L), sac-TMT combined with pembro solicits higher ORR, deep response rate, and duration of response compared to pembro monotherapy

ORR, Deep Response, and DOR (BICR)

Sac-TMT + pembro improved ORR, deep response rate, and duration of response vs. pembro



Source: Kelun-Biotech

I/O-TREATED NSCLC: ORR CONVERGENCE; IBI363 SHOWS POTENTIAL IN SURVIVAL TAIL

[First-in-class PD-1/IL-2^α-bispecific antibody IBI363 \(TAK-928\) in patients \(pts\) with advanced immunotherapy-resistant non-small cell lung cancer \(NSCLC\): Updated results from a phase I study. - ASCO](#)

[Efficacy and safety of HLX43 \(anti-PD-L1 ADC\) in patients with advanced non-small cell lung cancer. - ASCO](#)

ORR convergence despite diversified MoAs. ORRs cluster tightly regardless of drug class — B7-H3 ADCs, TROP2 ADCs, PD-L1 ADC, and bispecifics all land in a 22–37% band for the mature-sample cohorts. The outliers are histology-specific: HLX-43 from Henlius (not covered) leads in nsq-NSCLC at 47.4% (n=19, early-stage), while IBI363 leads in squamous at 36.7% (n=30) and also posts the longest mPFS in that histology at 10.1 months. In nsq, IBI363 sits at the lower end of the ORR range (24%).

IBI363 differentiates in 24m OS rate. IBI363 achieves a 47.8% 24-month OS rate in squamous and 42.7% in adenocarcinoma, compared to 8% and 21.7% respectively for docetaxel, and 29% for Dato-DXd in the adeno setting. IBI363 also records the longest mOS in both histologies (18.2 months in squamous, 17.5 months in adeno). This survival tail signal makes the upcoming Ph1b H2H against pembro+chemo exciting, where a direct comparison of survival against the SoC could offer IBI363 a chance to outrun competitors.

Safety — ADCs with the highest hematological burden. B7-H3 ADCs (HS-20093, YL201) post the heaviest Grade ≥3 TRAE burden driven by neutropenia (25.8–31.7% Grade ≥3) and anemia (17.2–25.0% Grade ≥3). HLX-43 sits in the middle at 42.9% Grade ≥3, while Ivonescimab + Docetaxel is lower at 30.2%. IBI363's Grade ≥3 TEAEs of 48.5% are not directly comparable given the different reporting convention (TEAE vs. TRAE), though haematological toxicity similarly dominates. No single class holds a commanding tolerability advantage.

EXHIBIT 12: **IBI363 differentiates in mOS and 48m OS rate**

								2026 ASCO		2026 ASCO
NSCLC ORR, %	B7H3 ADC				TROP2 ADC		Chemo	PD-L1 ADC	BsAb	
	HS-20093	I-DXd	YL201	DB-1311	Dato-DXd	Docetaxel	Docetaxel	HLX-43	Ivonescimab + Docetaxel	IBI363
Sponsor	Hansoh	Daiichi Sankyo	MediLink	Duality Bio	Daiichi Sankyo		BMS	Henlius	Akeso	Innovent
Trial	ARTEMIS-001 (NCT05276609)	NCT04145622	(NCT05434234 & NCT06057922)	NCT05914116	TROPION-Lung01 (NCT04656652)		CheckMate 017 CheckMate 057	(NCT06115642, NCT06907615)	NCT04736823	NCT05460767
Phase	I	I/II	I	I/IIa	III		III	I & II	II	I
Median line of previous therapies	2	3	2	2	1		1	2	1	2
Adeno AGA-										
Dosage	8mg/kg		2-2.8mg/kg	NA (not only AGA-)	6mg/kg (not only AGA-)	75mg/m2 (not only AGA-)	75mg/m2	2.5mg/kg	10/20mg/kg (AGA NM)	3mg/kg
Sample size	39		28	41	234	234	290	19	13	25
ORR, %	cORR: 33.3%		cORR: 28.6%	uORR: 22%	cORR: 31.2%	cORR: 12.8%	ORR: 12-13%	ORR: 47.4%	cORR: 30.8%	cORR: 24%
mPFS	7		4.2		5.5	3.6	4.3			4.2
24m OS rate %					29%	21.7%	16%			42.7%
mOS	17.5 (10mg/kg)				14.6	12.3	9.5			17.5 (low dose)
sq-NSCLC										
Dosage	8mg/kg	4.8-16mg/kg	2-2.8mg/kg	NA	6mg/kg	75mg/m2	75mg/m2	2mg/kg	10/20mg/kg	3mg/kg
Sample size	32	13	12	25	65	71	137	33	7	30
ORR, %	cORR: 15.6%	cORR: 31%	cORR: 8.3%	uORR: 16%	cORR: 9.2%	cORR: 12.7%	ORR: 9%	ORR: 36.4%	cORR: 57.1%	cORR: 36.7%
mPFS	4.1		4.1		2.8	3.9	2.8			10.1
24m OS rate %							8%			47.8%
mOS	8.8				7.6	9.4	6			18.2

Note: Daiichi Sankyo is covered by Miki Sogi, BMS is covered by Courtney Breen, MediLink and Henlius are not covered

Source: Company disclosure, ASCO, NEJM, Cell, Bernstein analysis

EXHIBIT 13: **B7H3 ADCs seem to have higher toxicity compared to others**

							2026 ASCO	2026 ASCO	
TRAE	B7H3 ADC		TROP2 ADC		Chemo		PD-L1 ADC	BsAb	
	HS-20093	YL201	Dato-DXd	Docetaxel	Docetaxel		HLX-43	Ivonescimab + Docetaxel	IBI363
Trial	ARTEMIS-001 (NCT05276609)	(NCT05434234 & NCT06057922)	TROPION-Lung01 (NCT04656652)		CheckMate-017	CheckMate-057	(NCT06115642, NCT06907615)	NCT04736823	NCT05460767
Any grade TRAEs									
Any grade TRAEs	100%	97.1%	87.5%	86.9%	86%	88%	97.1%	90.7%	99.3% (TEAE)
Anemia	75.3%	64.7%	14.8%	20.7%	22%	20%		13.9%	46.3% (TEAE)
Stomatitis			47.5%	15.5%	9%				
Neutropenia	65.6%	61.5%	4.7%	26.2%	33%	31%		16.3%	
Pneumonitis/ILD	2.2%	1.3%	8.8%	4.1%					
Grade ≥3 TRAEs									
Grade ≥3 TRAEs	51.6%	54.5%	25.6%	42.1%	55%	54%	42.9%	30.2%	48.5% (TEAE)
Anemia	17.2%	25.0%	4.0%	4.1%	3%	3%	12.2%	4.6%	4.4% (TEAE)
Stomatitis			6.7%	1.0%	0%				
Neutropenia	25.8%	31.7%	0.7%	23.4%	30%	27%	11.2%	9.3%	
Pneumonitis/ILD	0%	1.3%	3.7%	1.4%					

Source: Company disclosure, ASCO, NEJM, Cell, Bernstein analysis

B7H3 ADCs IN SCLC - NEW STARS FROM KELUN AND CSPC OFFER HIGH ORR AND DIFFERENTIATED SAFETY

[An open-label, first-in-human study of SKB500 in patients with locally advanced or metastatic solid tumors. - ASCO](#)

[First-in-human study of SYS6043, a novel B7-H3–targeting antibody-drug conjugate, in patients with advanced pan-tumor malignancies. - ASCO](#)

Unmet needs in small cell lung cancer (SCLC). SCLC represents 10–15% of all lung cancers globally and 350,000–400,000 new diagnoses per year. Despite being less prevalent than NSCLC, the disease burden is heavy: five-year survival rate is approximately 7% compared to 26% for NSCLC. 70% of patients diagnosed are already at extensive stage (ES-SCLC) beyond the help surgical options. The current 1L SoC for ES-SCLC is Pt chemo + PD-L1. For 2L and beyond, treatment options remain limited: lurbinectedin, topotecan, and irinotecan for Pt-resistant patients, and Pt re-treatment for Pt-sensitive ones. TCE tarlatamab was approved in 2024 for 2L+ ES-SCLC and reported 14.3m as the mOS, a meaningful step-up from the 10-11m range of various chemotherapies.

Two new contestants to the B7H3 ADC class. B7H3 overexpression in common SCLC - 60-80% SCLC tumors have moderate to high B7H3 surface expression, making it an ideal target. Front-runners in the B7H3 ADC class include HS-20093 (Hansoh) and DS-7300 (Daiichi Sankyo, covered by Miki Sogi / MSD, covered by Courtney Breen). Both CSPC and Kelun Biotech reported first-time results of their respective B7H3 ADCs this time. Notably ORR was high for both assets, SYS6043 at 64-75% and SKB500 at 71%, the highest in cross-trial comparison, and a 2X improvement to the SoC of lurbinectedin. Survival data are not yet mature.

On the safety front, both assets fared better than benchmark trials. SKB500 shows the lowest Grade ≥ 3 TRAE rate at 16.5%, with a 0% discontinuation/death rate — numerically best tolerability cross-trials. However, the comparatively small and early-phase cohort limits direct comparability.

Overall we see both Kelun-Biotech and CSPC as two serious new contestants in the B7H3 class despite their late entry, particularly differentiated on better safety profiles.

EXHIBIT 14: In 2L+ SCLC setting, Kelun's SKB500 and CSPC's SYS6043 showed encouraging ORRs

	SoCs			AACR 2025	Cell 2026		ASCO 2025	ASCO 2026		
Research Institute	PharmaMar	Amgen		Zai Lab	Hansoh		Daiichi Sankyo / MSD	Kelun Biotech	CSPC	
Product	Lurbinectedin	Tarlatamab		ZL-1310	Risvutalug rezetecan (HS-20093)		DS-7300	SKB500	SYS6043	
Target & Modality	Minor groove SMD	DLL3/CD3 BsAb		DLL3 ADC	B7H3 ADC		B7H3 ADC	B7H3 ADC	B7H3 ADC	
Trial No.	NCT02454972	DeLLphi-301, NCT05060016		NCT06179069	ARTEMIS-001, NCT05276609		IDeate-Lung01, NCT05280470	NCT06736327	ChiCTR2400094683	
Phase	II (Approved)	II		I	Ia/b		II	I	I/II	
Therapy type	2L	2L+		2L+	2L+		2L+	2L+	2L+	
Indication	ES-SCLC	ES-SCLC		ES-SCLC	ES-SCLC		ES-SCLC	SCLC	SCLC	
Patient characteristic	1 prior chemo	2 prior lines: 66% 3+ prior lines: 34% Brain mets: ~20% for 10mg, 36% for 100mg		1 prior line: 55.7% 2+ prior lines: 43.5% Brain mets: 32.2%	1 prior line: 40.6% 2 prior lines: 29.0% 3+ prior lines: 30.4% Brain mets: 14.5%		1 prior line: 23.4% 2 prior lines: 54.7% 3 prior lines: 21.9% Brain mets: 38%	≥ 2 prior lines: 34.5% prior platinum: 98.2% prior I/O: 76.4%		
Intervention	3.2 mg/m ² IV Q3W	10mg Q2W	100mg Q2W	Escalation (0.8mg/kg - 2.4mg/kg) + expansion IV Q3W	8mg/kg Q3W	10mg/kg Q3W	12mg/kg Q3W	12 mg/kg Q3W	6.0mg or 8.0mg/kg Q3W	6.0mg/kg Q3W
Number of enrollment	105	100	88	115	44	25	137	21	50	28
ORR %	35%	40%*	32%*	50%	54%*	50%*	48%*	71%	64%	75%
DCR %	65%	75%	64%	89%	84%	95%	88%	100%	92%	
DoR (months)	5.3	8.3	NE	6.1	7.1	8.9	5.3			
mPFS (months)	NE	4.9	3.9	5.4	5.9	7.3	4.9			
mOS (months)	10.8	14.3	NR	NM	10.1	16.3	10.3			

Note: NR stands for not reached, NM stands for not mature, NE stands for not estimated; * Confirmed ORR; Amgen and MSD are covered by Courtney Breen, PharmaMar is not covered

Source: Company disclosure, ASCO, Cell, NEJM, JCO, Bernstein analysis

EXHIBIT 15: In 2L+ ES-SCLC settings, Kelun's SKB500 showed the lowest TRAEs and ILD incidence

Target & modality	Minor groove SMD	DLL3 BsAb / ADC			B7-H3 ADCs				
Drug	Lurbinectedin	Tarlatabab		ZL-1310	HS-20093		DS-7300	SKB500	SYS6043
Trial	NCT02454972	DeLLphi-301		NCT06179069	ARTEMIS-001		IDeate-Lung01	NCT06736327	ChiCTR2400094683
Dose	3.2 mg/m ²	10mg	100mg	0.8mg/kg - 2.4mg/kg	8.0mg/kg	10.0mg/kg	12.0mg/kg	12.0mg/kg	6.0mg / 8.0mg/kg
Any grade TRAEs		88.7%	93.1%	83.5%	100.0%	100.0%	89.8%	72.2%	94.2%
Anemia				47.0%	93.2%	80.0%	34.3%	35.1%	52.0%
Nausea				27.8%	59.1%	44.0%	43.1%	34.0%	44.4%
Leukopenia / WBC decreased				20.9%	75.0%	88.0%	23.4%	24.7%	
Neutropenia				31.3%	59.1%	84.0%	34.3%		36.9%
Pneumonitis / ILD				8.7%	9.1%	8.0%	12.4%	2.1%	
Grade ≥3 TRAEs		25.6%	33.3%	20.0%	63.6%	72.0%	36.5%	16.5%	28.5%
Anemia				9.6%	15.9%	28.0%	10.2%	3.1%	
Nausea				0.0%	6.8%	4.0%	2.2%		
Leukopenia / WBC decreased				3.5%	20.5%	52.0%	3.6%		
Neutropenia				11.3%	25.0%	64.0%	13.9%		
Pneumonitis / ILD				1.7%	2.3%	0.0%	4.4%		
TRAEs led to treatment discount. or death		3.0%	3.4%	5.2%	15.9%	20.0%	13.9%	0.0%	

Source: ASCO, NEJM, ClinicalTrials.gov, Bernstein analysis

1L HR+/HER2- BC

Abstract 1066: BGB-43395 [First disclosure of frontline treatment \(1L tx\) with the selective CDK4 inhibitor BGB-43395 in combination with letrozole for metastatic HR+/HER2- breast cancer \(BC\): A phase 1 safety expansion. - ASCO](#)

BeOne released positive results for its fastest solid tumor pipeline BGB-43395. In 1L HR+/HER2- BC, BGB-43395 achieved an ORR of 68% in the 400mg BID group, and 58% and 40% for 240mg BID and 600mg BID group, respectively. The result is similar to its close competitor, Pfizer's CDK4i drug, atirromiclib, which gave 69.7% of ORR at 2025 ESMO.

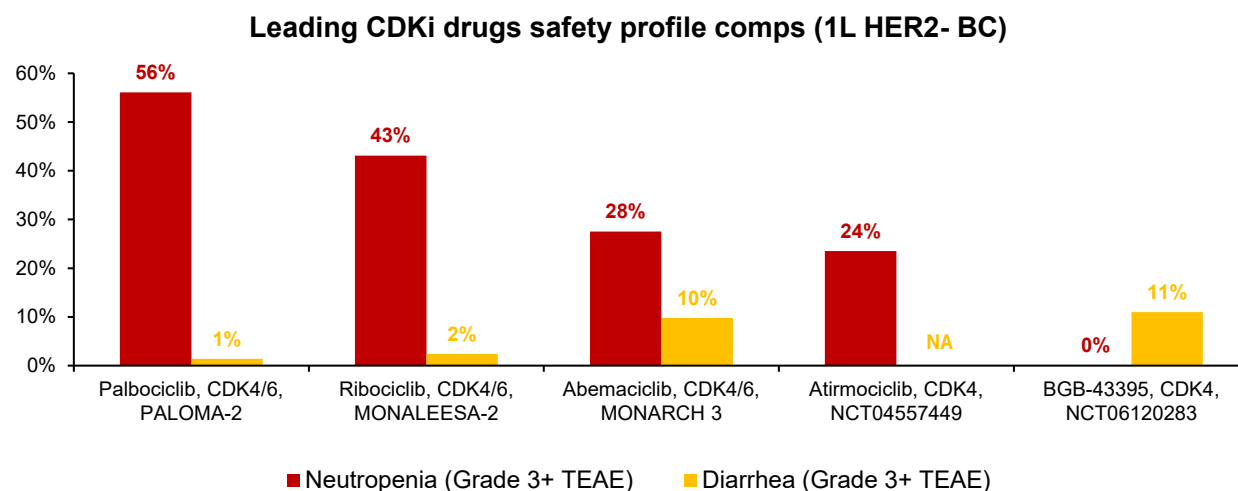
BGB-43395 also presented differentiated safety profile compared to last gen of CDK4/6i and atirromiclib. Grade 3 and above TEAE of neutropenia recorded 0%, indicating good hematological profile. Grade 3 and above TEAE in diarrhea appears higher than peers. However, company has indicated that G>3 TRAE of diarrhea is only 6% at fasted, and 0% when fed with food, so overall safety profile tolerable.

EXHIBIT 16: In HR+/HER2- breast cancer, BeOne reports ORR at 68% in the 400mg BID group, comparable to atimociclib

Sponsor	Amgen		Novartis		Lilly		Hengrui		Sino Biopharm		BeOne			Pfizer	
Drug	Palbociclib		Ribociclib		Abemaciclib		Dalpiciclib		Culmerciclib		BGB-43395			Atimociclib	
MoA	CDK4/6i		CDK4/6i		CDK4/6i		CDK4/6i		CDK2/4/6i		CDK4i			CDK4i	
Trial	PALOMA-2		MONALEESA-2		MONARCH 3		DAWNA-2		CULMINATE-2		NCT06120283			NCT04557449	
Efficacy Arms	Palbociclib + Letrozole		Ribociclib + Letrozole		Abemaciclib + fulvestrant		Dalpiciclib + letrozole		Culmerciclib + fulvestrant		BGB-43395 + letrozole			Atimociclib + letrozole	
Evaluable pts	444	222	334	334	328	165	303	153	290	142	19 (240mg BID)	19 (400mg BID)	20 (600mg BID)	34	
ORR	42.1%	34.7%	42.5%	28.7%	61.0%	45.5%	57.4%	47.7%	59.3%	42.3%	58%	68%	40%	69.7% (2025ESMO)	58.8% (2024 SABCS)
mPFS (m)	24.8	14.5	25.3	14.7	29.0	14.8	30.6	18.2	> 24.9	20.2					
PFS HR	0.58		0.56		0.54		0.51		0.56						
mOS (m)	53.9	51.2	63.9	51.4	66.8	53.7									
OS HR	0.96		0.76		0.8										

Note: Novartis is covered by Justin Smith, Eli Lilly and Pfizer are covered by Courtney Breen

Source: Journal of Clinical Oncology, Annals of Oncology, Pubmed, ASCO, company disclosure, Bernstein analysis

EXHIBIT 17: In 1L HR+/HER2- BC, BGB-43395 reports tolerable hematologic toxicities with G3 and above TEAE for neutropenia at 0%


For BGB-43395, G>3 TRAE of diarrhea is 6% at fasted, and 0% with food fed

Source: Company disclosure, ClinicalTrials.gov, Pharmcube, ASCO, Bernstein analysis

Nectin-4

Mabwell 9MW2821

[Bulmtatug fuvedotin \(BFv; 9MW2821\) plus toripalimab in perioperative patients with muscle-invasive bladder cancer \(MIBC\): Results of cohort A from a phase 2 study. - ASCO](#)

[Bulmtatug fuvedotin \(BFv; 9MW2821\) plus toripalimab in patients with locally advanced or metastatic urothelial carcinoma \(la/ muc\): Follow-up results from a phase 1b/2 study. - ASCO](#)

CSPC SYS6002

[A phase 1b study of SYS6002 in advanced solid tumors. - ASCO](#)

Hengrui SHR-A2102

[SHR-A2102, a Nectin-4 targeted antibody-drug conjugate, combined with HRS-4642 in previously treated pancreatic ductal adenocarcinoma harboring KRAS G12D mutation. - ASCO](#)

[Perioperative SHR-A2102, a novel nectin-4–targeted antibody-drug conjugate, in combination with adabrelimab for patients \(pts\) with muscle-invasive bladder cancer: Results from a phase 2/3 study. - ASCO](#)

[SHR-A2102 in combination with adabrelimab as first-line treatment in patients with locally advanced or metastatic squamous or non-squamous non-small cell lung cancer \(NSCLC\): Results from a phase 1b/2 study. - ASCO](#)

NECTIN4 ADCs in MIBC: comparable early pCR yet in small size cohorts. In muscle-invasive bladder cancer (MIBC), a high-risk setting with significant recurrence risk, perioperative systemic therapy is considered critical to improve long term outcomes. Historically, PD-(L)1 + chemo has been the SoC, achieving modest efficacy (pCR C.34%). More recently, PD-(L)1 + NECTIN4 ADCs (enfortumab vedotin + keytruda) has emerged as a new SoC with improved response. At ASCO 2026, Mabwell and Hengrui both reported early perioperative data for their NECTIN4 ADC combos, showing pCR rate of c.48-67%, broadly comparable to the EV + Keytruda regimen (Exhibit 18); however, these results are derived from relatively small and immature datasets (n=6 and 27 respectively) and require confirmation in larger randomized trials.

NECTIN4 ADCs in mUC: Padcev's success appears increasingly reproducible. In 1L & late-line metastatic urothelial carcinoma, data presented at ASCO 2026 from Mabwell and Hengrui's assets showed comparable efficacy vs. Padcev. In 1L setting, Mabwell's Bulumtaturg fuvodotin + toripalimab combo achieved an ORR of 83.0% and mPFS of 12.9m, compared with 67.7% (confirmed) and 12.5m for Padcev + keytruda. In late-line setting, CSPC's SYS6002 reported ORR of 41.0%, broadly in line with the 40.6% ORR reported for Padcev mono (Exhibit 19).

EXHIBIT 18: In perioperative MIBC settings, while early data from Mabwell and Hengrui's assets show comparable results to the SoC (EV + Pembro), larger sample sizes and more mature data are needed to determine efficacy differentiations

	SoC (cispatin-eligible)		SoC (cispatin-ineligible)		ASCO 2026	
	PD-(L)1 + chemo		NECTIN4 ADC + PD-1 combo			
	Durvalumab		Enfortumab Vedotin		Bulumtatug fuvedotin	SHR-A2102
Sponsor	AstraZeneca		MSD		Mabwell	Hengrui
Trial	NIAGARA, NCT03732677		KEYNOTE-B15, NCT04700124		NCT07314723	NCT06879145
Target modality	PD-L1 mAb		NECTIN4 ADC		NECTIN4 ADC	NECTIN4 ADC
LoT	Perioperative		Perioperative		Perioperative	Perioperative
Disease	Muscle-invasive bladder cancer (MIBC)		Muscle-invasive bladder cancer (MIBC)		Muscle-invasive bladder cancer (MIBC)	Muscle-invasive bladder cancer (MIBC)
Intervention (arm)	Durvalumab + GC	GC	EV + Keytruda	Chemo	Bulumtatug fuvedotin + toripalimab	SHR-A2102 + adebrelimab
Efficacy evaluable pts	533	530	405	403	6	27
pCR%	33.8%	25.8%	55.8%	32.5%	66.7%	48.1%
PDS%					83.3%	59.3%
mDFS (months)						
mEFS (months)	NR	46.1	NR	48.5		
EFS HR	0.68 ([0.56, 0.82], p<0.001)		0.53 ([0.41, 0.70], p<0.0001)			
mOS (months)						
OS HR	0.75 ([0.59, 0.93], p = 0.01)		0.65 ([0.48, 0.89], p = 0.0029)			
Safety profile	Grade ≥3 TRAEs: 40.6%	Grade ≥3 TRAEs: 40.9%	Grade ≥3 TRAEs: 45.5%	Grade ≥3 TRAEs: 45.9%		Grade ≥3 AEs: 40.5%

Note: AstraZeneca is covered by Justin Smith, Mabwell is not covered

Source: ASCO, NEJM, ClinicalTrials.gov, Bernstein analysis

EXHIBIT 19: In 1L and 2L+ mUC, Mabwell seems to achieve a high ORR and mPFS compared to padcev, with CSPC's SYS6002 showing comparable results with better safety

ASCO 2026					
	NECTIN4 ADC				
	Padcev		Bulumtatug fuvedotin	SYS6002	
Sponsor	Pfizer/Astellas		Mabwell	CSPC	
Trial	NCT03474107	NCT04223856	NCT06079112	ChiCTR2200066256	
Target modality	NECTIN4 ADC	NECTIN4 ADC	NECTIN4 ADC	NECTIN4 ADC	
LoT	2L+	1L	15% 2L+ patients 85% 1L patients	NM but likely 2L+	
Disease	aUC	aUC	Ia/mUC	aUC	
Intervention (arm)	Enfortumab vedotin	EV + pembrolizumab	BFv + toripalimab	SYS6002	
Efficacy evaluable pts	301	442	47	85	39 (RP2D)
ORR%	cORR: 40.6%	cORR: 67.7%	83.0%	37.6%	41.0%
mPFS (months)	5.55	12.5	12.9		
mOS (months)	12.88	31.5	18m OS rate: 68.1%	13.2	
Safety profile	Grade ≥3 TRAE: 51.4%	Grade ≥3 TRAE: 55.9% Disc: 35%	Grade ≥3 TRAE: 23.8% Death 0%	Disc: 1.3% Death 0%	

Note: Astellas is covered by Miki Sogi
Source: ASCO, NEJM, Bernstein analysis

DISCLOSURE APPENDIX

I. REQUIRED DISCLOSURES

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